



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

QUESTIONS

- Q1. A 55-year-old man presents with a pulsatile, compressible, soft midline neck swelling at the level of the hyoid bone that moves on BOTH swallowing AND tongue protrusion. It has been slowly enlarging. FNAC shows colloid and follicular cells. Diagnosis and treatment?



- a) Thyroid nodule — hemithyroidectomy
 - b) Thyroglossal cyst — Sistrunk's operation (excise cyst + central body of hyoid + tract to foramen caecum)
 - c) Branchial cyst — surgical excision
 - d) Dermoid cyst — simple excision under local anaesthetic
- Q2. A 25-year-old woman presents with a 3 cm lump at the angle of the mandible, anterior to the sternocleidomastoid muscle. It is fluctuant, transilluminable, and contains turbid fluid. FNA: squamous cells + cholesterol crystals + lymphocytes. No cervical lymphadenopathy. No ENT symptoms. What is the diagnosis and treatment?



- a) Metastatic squamous cell carcinoma — urgent panendoscopy
- b) Second branchial cleft cyst — surgical excision (formal neck dissection plane, preserving accessory nerve/IJV/SCM)
- c) Lymph node abscess — IV antibiotics
- d) Parotid Warthin's tumour

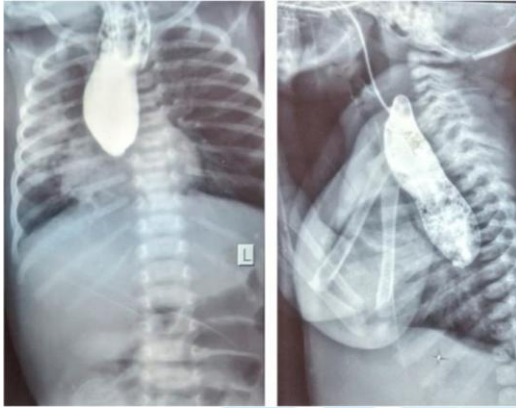
SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- Q3. A 35-year-old man has a 5 cm lipoma in the posterior triangle of the neck. It is soft, fluctuant, transilluminable, and multiloculated on MRI. It was present at birth. What is the MOST likely diagnosis?



- a) Lipoma — simple excision
 - b) Cystic hygroma (lymphangioma) — surgical excision ± sclerotherapy (doxycycline/bleomycin) for macrocystic lesions
 - c) Branchial cyst — surgical excision
 - d) Reactive lymphadenopathy — antibiotics
- Q4. A 2-day-old neonate has been unable to feed and passes frothy saliva. Nasogastric tube coils back in the oropharynx. CXR shows the NGT coiled in the upper oesophagus with gastric air present below the diaphragm. What type of oesophageal atresia (EA) and tracheo-oesophageal fistula (TOF) is this?

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) Type A (pure EA) — no fistula
- b) Type C (EA + distal TOF) — most common (85%), gas present in stomach
- c) Type B (EA + proximal TOF)
- d) Type E (H-type TOF, no EA)

Q5. A 6-week-old male infant born at term presents with a history of bilious vomiting since day 1 of life. He has not been gaining weight. AXR shows a 'double bubble' with NO distal gas. USS shows 'snowstorm' appearance of the duodenum. What is the MOST likely diagnosis and associated condition?



- a) Pyloric stenosis — associated with first-born male, Ramstedt's pyloromyotomy
- b) Duodenal atresia — associated with Down syndrome (trisomy 21) in 30%
- c) Annular pancreas — ring of pancreatic tissue around 2nd part of duodenum
- d) Malrotation with midgut volvulus — whirlpool sign on USS

Q6. A 3-year-old boy presents with painless rectal bleeding and anaemia. Technetium-99m pertechnetate scan (Meckel's scan) is positive. What is the RULE OF 2s for Meckel's diverticulum?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) 2 cm long, 2 feet from IC valve, 2% of population, presents in 1st 2 years, 2 types of ectopic tissue
- b) 2 inches long, 2 feet from IC valve, 2% of population, 2× more common in males, presents in 1st 2 years, 2 types of ectopic tissue
- c) 2% of population, 2 cm from IC valve, presents at 2 months of age
- d) 2 types of symptoms, 2 cm long, 2 feet from duodenum

Q7. A 4-year-old girl presents with bilious vomiting, distension, and failure to thrive since birth. AXR shows dilated loops with no rectal gas. Contrast enema shows a very short aganglionic segment confined to the anus and distal rectum. Biopsy confirms absence of ganglion cells. Which of the following is TRUE regarding surgical management?



- a) Conservative management with bowel irrigation only — short segment Hirschsprung's can be managed non-operatively indefinitely
- b) Definitive pull-through procedure (Swenson, Soave, or Duhamel) is the treatment — brings ganglionated bowel to the anus
- c) Permanent colostomy is the only surgical option
- d) Anorectal myectomy is sufficient for all cases of short-segment Hirschsprung's



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- Q8. A neonate is born with a midline abdominal wall defect directly through the umbilical cord insertion. The defect contains liver, bowel, and stomach covered by a sac (peritoneum + amnion). Which of the following is CORRECT?



- a) This is gastroschisis — bowel is uncovered, lateral to umbilicus, no liver herniation
 - b) This is exomphalos (omphalocele) — defect at umbilicus, covered by sac; 50% have associated chromosomal/cardiac anomalies
 - c) This is an umbilical hernia — reducible, benign, closes spontaneously by 5 years
 - d) This is prune belly syndrome — abdominal wall aplasia with GU abnormalities
- Q9. A 45-year-old woman with gastric outlet obstruction has: Na^+ 128, K^+ 2.8, Cl^- 78, HCO_3^- 38, pH 7.58, urine $\text{Cl}^- < 10$. What explains paradoxical aciduria?
- a) Kidney excretes HCO_3^- to correct alkalosis
 - b) Severe K^+ depletion forces Na^+/H^+ exchange in distal tubule
 - c) Aldosterone promotes H^+ excretion preferentially
 - d) Cl^- deficit stimulates H^+ retention in proximal tubule
- Q10. Todani Type IV choledochal cyst — which of the following is CORRECT?
- a) Single fusiform extrahepatic cyst — most common type
 - b) Multiple cysts involving both intra and extrahepatic bile ducts — treated with liver transplant or resection
 - c) Intraduodenal choledochoceles — managed with ERCP sphincterotomy
 - d) Intrahepatic cysts only — same as Caroli disease



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q11. A patient undergoes emergency laparotomy for small bowel obstruction.

Operative finding:

- True diverticulum on antimesenteric border
- 60 cm proximal to ileocecal junction

Embryological origin?

- a) Vitellointestinal duct
- b) Urachus
- c) Omphalomesenteric artery
- d) Neural crest

Q12. A barium swallow demonstrates a large posterior pharyngeal pouch filling with contrast above the upper esophageal sphincter.

Most likely diagnosis?



- a) Achalasia cardia
- b) Zenker diverticulum
- c) Diffuse esophageal spasm
- d) Barrett esophagus

Q13. A patient develops severe dysphagia.

Barium swallow shows:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



"Corkscrew esophagus"

Diagnosis?

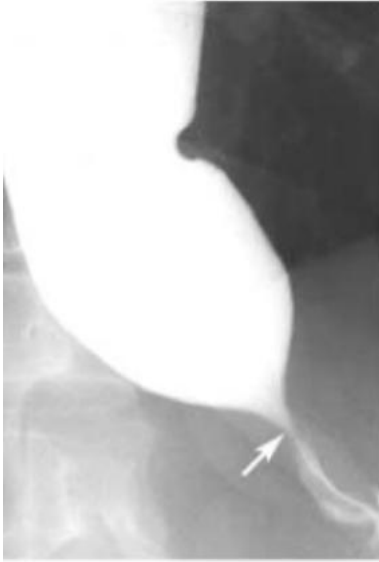
- a) Achalasia
- b) Diffuse esophageal spasm
- c) Esophageal cancer
- d) Barrett esophagus

Q14. A 38-year-old woman has progressive dysphagia to both solids and liquids for 2 years with nocturnal regurgitation. Manometry shows failure of LES relaxation and absent peristalsis. She undergoes laparoscopic Heller myotomy.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



Which additional procedure is usually added to reduce postoperative reflux?

- a) Total gastrectomy
- b) Partial fundoplication
- c) Billroth II reconstruction
- d) Pyloroplasty

Q15. A 55-year-old patient with longstanding GERD develops Barrett esophagus.



Biopsy:
High-grade dysplasia.

Best management?

- a) Observation



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- b) Long-term PPI only
- c) Endoscopic removal
- d) Antacids

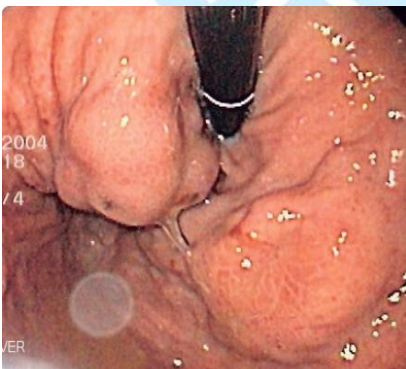
Q16. A 34-year-old man has sudden severe chest pain after forceful vomiting. He is toxic, febrile, and has subcutaneous emphysema. Chest X-ray shows left pleural effusion.



Most likely diagnosis?

- a) Mallory-Weiss tear
- b) Boerhaave syndrome
- c) Achalasia cardia
- d) Zenker diverticulum

Q17. A 49-year-old man with chronic pancreatitis develops recurrent upper GI bleeding. Endoscopy shows isolated gastric varices without esophageal varices. Liver function tests are normal.



Best explanation?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Portal vein thrombosis from cirrhosis
- b) Splenic vein thrombosis causing left-sided portal hypertension
- c) Hepatic vein thrombosis
- d) Mallory-Weiss tear

Q18. A 60-year-old man with acute pancreatitis develops persistent fever and leukocytosis after 2 weeks. CECT shows walled-off pancreatic necrosis with gas bubbles.



Best management strategy?

- a) Immediate open necrosectomy in all cases
- b) Step-up approach with antibiotics and image-guided drainage first
- c) Total pancreatectomy
- d) Observation only

Q19. A 44-year-old woman presents with severe epigastric pain radiating to the back. Serum lipase is elevated. She has gallstones on ultrasound. MRCP shows CBD stone.



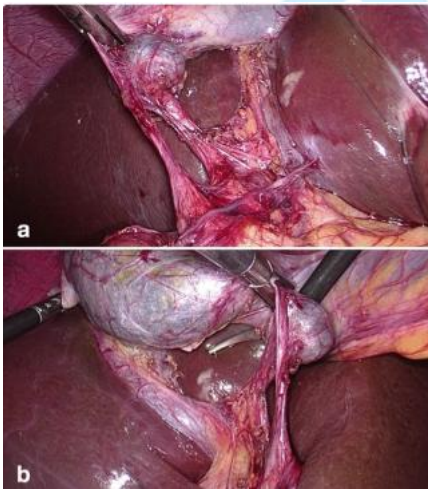
Most appropriate definitive management after stabilization?

- a) ERCP for CBD clearance followed by cholecystectomy

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- b) Observation without surgery
- c) Distal pancreatectomy
- d) Total gastrectomy

Q20. During laparoscopic cholecystectomy, the surgeon wishes to achieve the "critical view of safety" (CVS). What are the TWO criteria that must be satisfied?



- a) Cystic artery and cystic duct visualised entering the gallbladder + Calot's triangle cleared of fat
- b) Only the cystic duct needs to be visualised entering the gallbladder
- c) Two structures seen entering the gallbladder + hepatocystic triangle cleared of fat/fibrous tissue
- d) Common bile duct and cystic duct identified separately

Q21. Which of the following is the CORRECT description of the Hasson technique for laparoscopic port insertion?



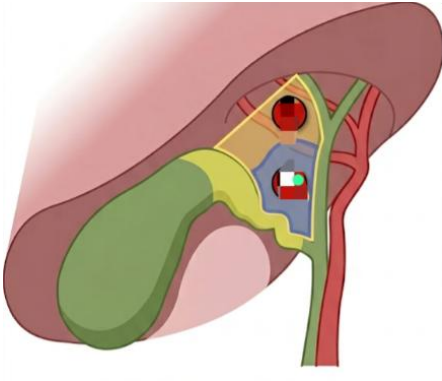
- a) Veress needle inserted blindly at umbilicus to create pneumoperitoneum
- b) Open cut-down to peritoneum under direct vision at umbilicus — avoids blind entry
- c) Optical trocar with transparent tip inserted under vision without open cut-down
- d) Direct trocar entry without pneumoperitoneum at Palmer's point



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q22. Which of the following correctly defines the boundaries of Calot's triangle?



- a) Cystic duct (inferior) + common hepatic duct (medial) + cystic artery (superior)
- b) Cystic duct (inferior) + common hepatic duct (medial) + inferior surface of liver (superior)
- c) Cystic duct + CBD + right hepatic artery
- d) Portal vein + CBD + cystic duct

Q23. A 52-year-old woman is found to have a 12mm gallbladder polyp on USS incidentally. She is asymptomatic. What is the correct management?



- a) Discharge — all polyps <15mm are benign
- b) 6-monthly USS surveillance for 2 years
- c) Laparoscopic cholecystectomy — polyps ≥ 10 mm require surgery
- d) EUS first to characterise, then decide

Q24. A 55-year-old woman presents with Mirizzi syndrome Type II (stone eroding into CBD creating a cholecystocholodochal fistula). What is the MOST appropriate surgical approach?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Simple laparoscopic cholecystectomy with cystic duct ligation
- b) Subtotal cholecystectomy + repair of fistula with T-tube or Roux-en-Y hepaticojejunostomy depending on defect size
- c) ERCP sphincterotomy and stone extraction only
- d) Whipple's procedure

Q25. What is the mechanism of gallstone formation in patients with haemolytic anaemia (e.g., sickle cell disease, hereditary spherocytosis)?

- a) Cholesterol supersaturation of bile
- b) Excess unconjugated bilirubin precipitation forming calcium bilirubinate (black pigment stones)
- c) Bile stasis and infection with *E. coli* producing β -glucuronidase (brown pigment stones)
- d) Mucin hypersecretion creating a nidus for cholesterol crystal aggregation

Q26. A 35-year-old man presents with fever, chills, and jaundice. MRCP shows multiple areas of intrahepatic and extrahepatic bile duct narrowing alternating with dilation (beading pattern). He has a 15-year history of ulcerative colitis. Biopsy shows periductal 'onion-skin' fibrosis. Diagnosis?



- a) Primary biliary cholangitis
- b) IgG4-related cholangitis
- c) Primary sclerosing cholangitis
- d) Caroli disease



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

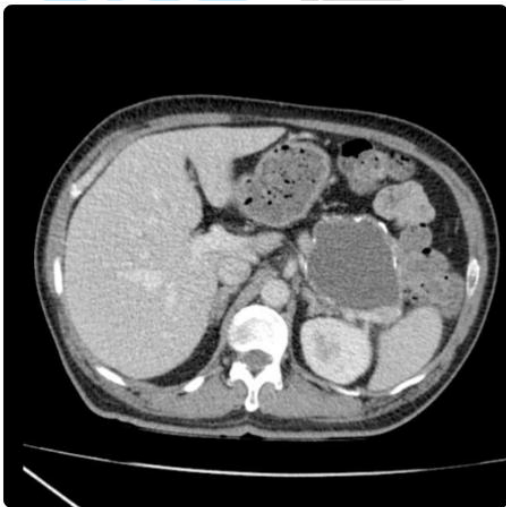
Q27. According to Strasberg classification, which type of bile duct injury involves a complete excision (clip and cut) of the common bile duct?

- a) Type A — cystic stump leak
- b) Type C — aberrant right duct leak
- c) Type E — complete transection/excision equivalent to Bismuth classification
- d) Type D — lateral injury

Q28. A 45-year-old alcoholic develops:

- Persistent abdominal pain
- Early satiety
- Vomiting

CT:



- 8-cm cystic lesion
- Adjacent to pancreas
- Well-defined wall
- Present 8 weeks after pancreatitis

Most likely diagnosis?

- a) True pancreatic cyst
- b) Pseudocyst
- c) Mucinous cystadenoma
- d) Serous cystadenoma



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q29. A 42-year-old female presents with severe RUQ pain, fever, and jaundice.

What is this triad called?

- a) Beck triad
- b) Charcot triad
- c) Virchow triad
- d) Cushing triad

Q30. A 48-year-old chronic alcoholic presents with recurrent disabling epigastric pain radiating to the back for 8 years. He has lost 15 kg weight and has developed steatorrhea. CT demonstrates:



- Multiple intraductal calcifications
- Main pancreatic duct diameter 11 mm
- No inflammatory mass in pancreatic head
- No biliary obstruction

Despite pancreatic enzyme supplementation and complete abstinence from alcohol for 18 months, symptoms persist.

Which operative strategy is most appropriate?

- a) Pancreaticoduodenectomy (Whipple)
- b) Longitudinal pancreaticojejunostomy
- c) Distal pancreatectomy with splenectomy
- d) Total pancreatectomy

Q31. A 42-year-old female develops severe abdominal pain 24 hours after ERCP.

Findings:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- Serum lipase 1500 U/L
- CT: pancreatic enlargement with peripancreatic fluid

Most important risk factor for post-ERCP pancreatitis?

- a) Male sex
- b) Previous cholecystectomy
- c) Sphincter of Oddi dysfunction
- d) Age >60 years

Q32. A 68-year-old man presents with acute severe abdominal pain and distension. CT shows cecal volvulus with ischemic bowel.



Best management?

- a) Colonoscopic decompression as first-line definitive therapy
- b) Emergency right hemicolectomy
- c) Elective sigmoid colectomy
- d) Conservative management only

Q33. A 35-year-old man presents to the ED with sudden onset severe right loin-to-groin pain, nausea, and vomiting. He is afebrile. Urine dipstick shows haematuria. CT KUB reveals a 4mm stone at the right vesicoureteric junction (VUJ). eGFR is normal. Contralateral kidney is normal. What is the MOST appropriate management?

- a) Emergency ureteroscopy and laser lithotripsy

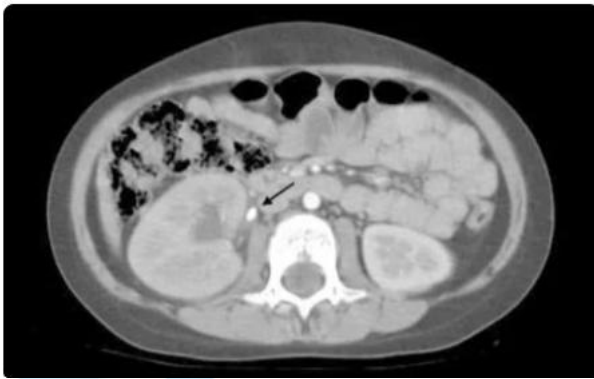


MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- b) Medical expulsive therapy — tamsulosin 0.4mg OD + NSAIDs; trial for 4 weeks with follow-up CT KUB
- c) ESWL (extracorporeal shock wave lithotripsy) immediately
- d) Percutaneous nephrostomy to decompress the right kidney

Q34. A 45-year-old woman presents with right flank pain and fever (38.9°C). CT KUB shows a 9mm stone at the right pelviureteric junction (PUJ) causing complete obstruction of the right kidney with moderate hydronephrosis. WBC is 18,000. She is tachycardic (HR 115). Urine culture is pending. What is the MOST urgent management?



- a) IV antibiotics + analgesia and wait 48 hours for culture results before drainage
- b) Emergency ureteroscopy and stone removal immediately
- c) Urgent drainage — ureteric stent (JJ stent) OR percutaneous nephrostomy + IV broad-spectrum antibiotics; stone treatment deferred
- d) ESWL for the obstructing stone + oral antibiotics

Q35. A 50-year-old obese man has a 15mm stone in the right renal pelvis found on CT. He is asymptomatic. The stone is radio-opaque on plain X-ray. Renal function is normal. No infection. Which treatment modality is MOST appropriate?

- a) Medical expulsive therapy — 15mm stones pass spontaneously
- b) ESWL (extracorporeal shock wave lithotripsy) — first-line for renal pelvis stones 10–20mm
- c) Percutaneous nephrolithotomy (PCNL) — gold standard for stones >20mm
- d) Flexible ureteroscopy and laser lithotripsy (FURS/RIRS) — endoscopic alternative for 10–20mm renal stones

Q36. A 28-year-old woman with a known single right kidney presents to the ED with left loin pain and fever (39.1°C). CT KUB shows a 6mm stone at the left PUJ causing complete left



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

hydronephrosis. She is haemodynamically stable. What additional factor makes this case a SURGICAL EMERGENCY that would not apply if she had two kidneys?

- a) Her age — young patients require faster treatment
- b) Obstruction of a SOLITARY FUNCTIONING KIDNEY requires IMMEDIATE decompression regardless of infection status — risk of permanent renal failure
- c) 6mm stones always require ureteroscopy
- d) Female sex — women require faster stone treatment

Q37. A 40-year-old man presents with a 22mm stone filling the right renal pelvis and upper calyx (staghorn calculus). It is radio-opaque. Culture grows *Proteus mirabilis*. Renal function is normal. He has no previous urological surgery. What is the GOLD STANDARD treatment?



- a) ESWL — non-invasive, preferred for all stone sizes
- b) Long-term antibiotics — infection stones respond to antibiotics
- c) Percutaneous nephrolithotomy (PCNL) — gold standard for stones >20mm and staghorn calculi
- d) Open nephrolithotomy — all infection stones require open surgery

Q38. A 32-year-old man has recurrent bilateral renal calculi since childhood. His stones are radio-opaque and have a hexagonal shape on microscopy. 24-hour urine shows: cystine 900mg/24h (normal <50mg). Serum amino acids show elevated cystine, ornithine, lysine, arginine. What is the diagnosis, and what is the SPECIFIC medical treatment?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) Calcium oxalate stones — thiazide diuretics + low oxalate diet
- b) Cystinuria — high fluid intake (>3L/day) + urinary alkalinisation (pH >7.5) + tiopronin (alpha-mercaptopropionylglycine) or D-penicillamine for severe cases
- c) Uric acid stones — potassium citrate + allopurinol
- d) Struvite stones — antibiotics + PCNL

Q39. A 60-year-old man presents with painless gross haematuria. CT urogram shows a 2cm stone in the right renal pelvis WITH an underlying soft tissue filling defect beneath the stone (visible on delayed phase). He is a heavy smoker. What is the MOST important next step?

- a) Treat the stone with ESWL — haematuria will resolve
- b) Ureteroscopy — inspect the urothelium beneath the stone for urothelial carcinoma; stone treatment secondary to cancer exclusion
- c) Cystoscopy only — to exclude bladder cancer
- d) Nephrectomy immediately — assume malignancy

Q40. A 45-year-old man presents with a sudden onset of severe left flank pain radiating to the left groin, nausea, and haematuria. CT KUB shows a 5mm stone at the left vesicoureteric junction (VUJ). eGFR is normal. He is haemodynamically stable. What is the MOST appropriate management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) Emergency ureteroscopy — all stones require immediate removal
- b) Conservative management (alpha-blocker — tamsulosin + analgesia) — stones $\leq 6\text{mm}$ at VUJ have 70% spontaneous passage rate with alpha-blockers
- c) ESWL (extracorporeal shock wave lithotripsy) — first-line for all ureteric stones
- d) Emergency nephrostomy to drain the obstructed kidney

Q41. A 68-year-old man presents with acute large bowel obstruction. X-ray abdomen shows massively dilated colon with a coffee-bean appearance arising from the pelvis. Sigmoidoscopy reveals viable mucosa and successful decompression is achieved.



Next best management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Discharge after decompression
- b) Elective sigmoid colectomy after stabilization
- c) Right hemicolectomy
- d) Emergency total colectomy

Q42. A 22-year-old male develops peri-umbilical pain that migrates to the right iliac fossa after 12 hours. He has rebound tenderness at McBurney point.

Most likely diagnosis?

- a) Crohn disease
- b) Acute appendicitis
- c) Meckel diverticulitis
- d) Cecal carcinoma

Q43. A 22-year-old man presents with 24 hours of periumbilical pain that has migrated to the right iliac fossa. Temperature is 38.1°C, WBC 14,200, CRP 68 mg/L. On examination, there is localised tenderness at McBurney's point with guarding. Rovsing's sign is positive. His Alvarado score is 8. CT scan is unavailable. What is the MOST appropriate next step?

- a) Discharge with oral antibiotics and review in 48 hours
- b) Diagnostic laparoscopy before any decision
- c) Emergency appendicectomy — Alvarado score ≥ 7 warrants surgery without CT in males
- d) USS abdomen to confirm diagnosis before surgery

Q44. A 28-year-old woman presents with 36 hours of RIF pain, fever (38.6°C), and vomiting. On examination, there is a tender mass palpable in the RIF. WBC 18,500, CRP 210 mg/L. CT abdomen shows a well-defined 6×5 cm inflammatory mass in the RIF with the appendix at its centre, surrounded by omentum and loops of bowel, with no drainable fluid collection. What is this clinical entity and the MOST appropriate management?

- a) Appendicular abscess — CT-guided drainage + antibiotics
- b) Appendicular mass (phlegmon) — non-operative management with IV antibiotics, bowel rest, analgesia; interval appendicectomy at 6–8 weeks
- c) Perforated appendicitis with faecal peritonitis — emergency laparotomy immediately
- d) Caecal carcinoma — urgent colonoscopy before surgery



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q45. A 55-year-old man presents with a 5-day history of RIF pain and fever. CT shows a 4×3 cm rim-enhancing fluid collection adjacent to the appendix tip with surrounding fat stranding. No free air. WBC 22,000, CRP 280 mg/L. He is haemodynamically stable. What is the DIAGNOSIS and BEST management?



- a) Appendicular mass (phlegmon) — non-operative Ochsner-Sherren regimen
- b) Appendicular abscess — CT/USS-guided percutaneous drainage + IV antibiotics; appendicectomy deferred
- c) Perforated appendicitis with peritonitis — immediate laparotomy
- d) Crohn's disease ileitis — mesalazine and steroids

Q46. A 30-year-old man undergoes laparoscopic appendicectomy. The appendix is removed. Post-operative histology reports a 2.5cm well-differentiated neuroendocrine tumour (NET/carcinoid) at the tip of the appendix with clear resection margins. Lymphovascular invasion is absent. Depth of invasion: mesoappendix (<3mm). What is the NEXT step in management?

- a) No further surgery — appendicectomy is curative for NETs ≤ 2 cm; histology in this case requires right hemicolectomy
- b) Right hemicolectomy — all appendiceal NETs require right hemicolectomy
- c) Right hemicolectomy indicated for NETs >2cm; 2.5cm tumour with mesoappendix invasion <3mm = right hemicolectomy
- d) Octreotide long-acting therapy — all appendiceal NETs require somatostatin analogues

Q47. A 40-year-old man presents with a 3-day history of RIF pain and low-grade fever. His CRP is 180 mg/L and WBC is 16,000. CT shows a perforated appendix with generalised free fluid and air in the peritoneal cavity with faecal peritonitis. He is haemodynamically stable. What is the MOST appropriate surgical management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Non-operative Ochsner-Sherren regimen — all perforated appendicitis can be managed conservatively
- b) Emergency laparoscopic appendicectomy + peritoneal lavage + drain
- c) Emergency laparotomy — laparoscopic approach contraindicated in perforated appendicitis with faecal peritonitis
- d) CT-guided percutaneous drainage only

Q48. A 22-year-old male presents with acute appendicitis. During laparoscopic appendectomy, a normal appendix is visualized.

What should be the next step?

- a) Close abdomen without further exploration
- b) Remove appendix only
- c) Systematically inspect terminal ileum, Meckel's diverticulum and pelvic organs
- d) Convert to open surgery immediately

Q49. A 65-year-old man develops postoperative intestinal obstruction 4 days after colectomy.

Most likely cause?

- a) Colon cancer recurrence
- b) Adhesions
- c) Volvulus
- d) Intussusception

Q50. A 42-year-old man presents with severe abdominal pain out of proportion to physical findings. He has atrial fibrillation. Serum lactate is elevated. CT angiography shows embolic occlusion of the superior mesenteric artery.

Most likely segment of bowel first affected?

- a) First part of duodenum
- b) Jejunum and ileum
- c) Rectum
- d) Distal sigmoid colon



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

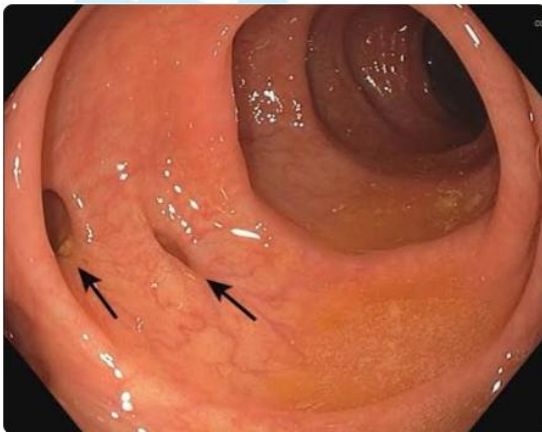
SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q51. A 67-year-old man develops abdominal pain after meals and significant weight loss over 6 months. He avoids eating because of pain. CT angiography shows severe atherosclerotic narrowing of the celiac and superior mesenteric arteries.

Diagnosis?

- a) Acute mesenteric ischemia
- b) Chronic mesenteric ischemia
- c) Ogilvie syndrome
- d) Adhesive intestinal obstruction

Q52. A 72-year-old man presents with massive painless lower GI bleeding. Colonoscopy after stabilization shows multiple outpouchings in sigmoid colon. No active malignancy is seen.



Most likely cause?

- a) Diverticulosis
- b) Crohn disease
- c) Ulcerative colitis
- d) Ischemic colitis

Q53. A 58-year-old man presents with a sudden onset painless bright red rectal bleeding, passing approximately 200 mL of blood. Colonoscopy is performed after bowel preparation — reveals multiple diverticula throughout the colon with active bleeding from a diverticulum in the sigmoid colon. What is the FIRST-LINE treatment?

- a) Emergency sigmoid colectomy immediately
- b) Endoscopic haemostasis (injection/clipping/thermal) at colonoscopy — first-line for diverticular bleeding
- c) Angioembolisation — superior to colonoscopic haemostasis for diverticular bleeding



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- d) Watchful waiting — 80% of diverticular bleeding stops spontaneously

Q54. A 78-year-old hospitalized patient with pneumonia develops progressive abdominal distension. X-ray shows massive colonic dilatation, especially cecum, without mechanical obstruction. CT excludes volvulus and tumor.



Most likely diagnosis?

- a) Sigmoid volvulus
- b) Ogilvie syndrome
- c) Toxic megacolon
- d) Hirschsprung disease

Q55. A 28-year-old woman with ulcerative colitis has severe disease refractory to steroids and biologics. She develops dysplasia on surveillance colonoscopy. Which operation is most definitive?

- a) Segmental colectomy
- b) Total proctocolectomy with ileal pouch-anal anastomosis
- c) Right hemicolectomy
- d) Strictureplasty

Q56. A 72-year-old man presents with progressive urinary frequency, nocturia $\times 4$, poor stream, and post-void dribbling. International Prostate Symptom Score (IPSS) is 24 (severe). PSA is 3.2 ng/mL. DRE shows a symmetrically enlarged, smooth prostate. Residual urine volume is 250 mL. What is the MOST appropriate treatment?

- a) Watchful waiting — LUTS in elderly men do not need treatment



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- b) Alpha-blocker (tamsulosin) alone — first-line for all LUTS severity
- c) Combination alpha-blocker + 5-alpha reductase inhibitor (dutasteride) for large gland (>40mL) with severe LUTS + TURP if fails medical management
- d) Immediate TURP — surgery is first-line for severe LUTS

Q57. A 72-year-old man presents with progressive LUTS for 2 years: hesitancy, poor stream, terminal dribbling, nocturia, and incomplete emptying. He now develops acute urinary retention. DRE shows a smooth, enlarged, rubbery prostate. PSA is mildly elevated. After catheterization, he develops fever, chills, and suprapubic pain.

Which statement is most appropriate?

- a) Smooth enlargement is diagnostic of prostate carcinoma
- b) Acute bacterial prostatitis should be treated with prostatic massage
- c) BPH commonly causes both voiding and storage symptoms
- d) TURP-guided biopsy is the first step in acute prostatitis

Q58. A 45-year-old man is involved in a head-on collision. He has a seat belt sign (bruising across the lower abdomen/lap), abdominal tenderness, and a high-riding prostate on rectal examination. There is blood at the urethral meatus. What is the CORRECT initial step for urological assessment?

- a) Insert urethral catheter immediately to measure urine output
- b) Perform retrograde urethrogram (RUG) before any catheterisation — blood at meatus = urethral injury until excluded
- c) Suprapubic catheter immediately
- d) CT cystogram first

Q59. A 72-year-old male presents with:

- Hesitancy
- Weak stream
- Nocturia

DRE:
Smooth enlarged prostate.

Most likely diagnosis?

- a) Prostate cancer
- b) BPH



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- c) Prostatitis
- d) Urethral stricture

Q60. A 28-year-old man is brought to the ED following a road traffic accident. He sustained a pelvic fracture (pubic symphysis diastasis + bilateral pubic rami fractures). On examination, he has blood at the urethral meatus, inability to void, and a high-riding prostate on digital rectal examination. His bladder is palpably distended. What is the CORRECT sequence of management?

- a) Insert urethral catheter immediately to drain the bladder
- b) Retrograde urethrogram (RUG) first to confirm and characterise the urethral injury; if injury confirmed → suprapubic catheter (SPC) insertion
- c) CT pelvis with contrast first — defines both the pelvic fracture and urethral injury simultaneously
- d) Immediate cystoscopy to visualise the urethra under direct vision

Q61. Following the retrograde urethrogram, a complete posterior urethral disruption is confirmed in the 28-year-old man from Q1. He has a stable pelvic fracture. He is haemodynamically stable. What is the MOST appropriate immediate management of the urethral injury?

- a) Immediate primary end-to-end anastomotic urethroplasty in the acute setting
- b) Suprapubic catheter for urinary diversion; delayed urethroplasty at 3–6 months
- c) Immediate urethral realignment over a catheter (primary realignment) within 24–48 hours
- d) Permanent urinary diversion (ileal conduit) — complete disruptions cannot be repaired

Q62. A 35-year-old man presents after a fall astride a metal fence railing while cycling. He has perineal pain, haematuria, and swelling of the perineum. Examination shows bruising and oedema of the perineum. He is unable to void. Blood is visible at the urethral meatus. RUG shows extravasation of contrast below the urogenital diaphragm, specifically in the region of the bulbar urethra. Which urethral injury is this?

- a) Posterior urethral injury — prostatic urethra disruption
- b) Posterior urethral injury — membranous urethra disruption
- c) Anterior urethral injury — bulbar urethra injury from straddle trauma
- d) Anterior urethral injury — penile urethra laceration

Q63. A 50-year-old man presents with a large right inguinal hernia containing bowel that is irreducible. He has colicky abdominal pain, vomiting, and distension. The hernia is tense and tender. What is the surgical finding that would mandate bowel resection?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Bowel appearing dark purple-black despite releasing obstruction (irreversible ischaemia)
- b) Bowel appearing dusky but pinking up after releasing obstruction
- c) 'Serositis' (inflamed but viable serosa)
- d) Small amount of serosanguinous fluid in the hernia sac

Q64. A 45-year-old man presents with a right inguinal hernia that has been present for 5 years. On examination, the hernia is easily reducible and asymptomatic. He is otherwise fit. Which of the following is the MOST appropriate management?

- a) Emergency repair — all inguinal hernias should be repaired immediately
- b) 'Watchful waiting' is safe for asymptomatic reducible inguinal hernias — repair when symptomatic or complicated
- c) Laparoscopic repair only — open repair is obsolete
- d) Truss (hernia belt) as definitive long-term management

Q65. A 65-year-old woman presents with a tender, irreducible lump BELOW and LATERAL to the pubic tubercle, in the right groin. It is 2 cm, non-pulsatile. There is mild erythema overlying the skin. What is the diagnosis and URGENCY?

- a) Direct inguinal hernia — elective repair in 6 weeks
- b) Femoral hernia — urgent repair (high strangulation risk — McEvedy, Lockwood, or laparoscopic approach)
- c) Lymph node — fine needle aspiration
- d) Saphena varix — compression

Q66. A Richter's hernia is best described as:

- a) A hernia containing two loops of bowel forming a W-shape (Maydl's hernia)
- b) Only the antimesenteric wall of the bowel herniates — bowel lumen not completely obstructed, but strangulation is possible
- c) A hernia containing Meckel's diverticulum (Littre's hernia)
- d) A sliding hernia where the caecum forms part of the hernia sac wall

Q67. A 48-year-old man presents with a 3 cm midline scar in the upper abdomen. He has a dragging sensation after eating. USS shows a small defect in the linea alba with omentum protruding. What is this hernia and management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Umbilical hernia — mesh repair if symptomatic
- b) Epigastric hernia — repair with mesh (Lichtenstein-equivalent midline closure) if symptomatic
- c) Paraumbilical hernia — requires urgent repair
- d) Spigelian hernia — lateral abdominal wall defect

Q68. A 62-year-old obese man presents with an irreducible right groin swelling for 10 hours. He has severe pain, vomiting, abdominal distension, and absolute constipation. On examination, the swelling is tense, tender, and extends into the scrotum. Cough impulse is absent. Skin over the swelling is normal.

What is the most appropriate management?

- a) Attempt repeated manual reduction under sedation
- b) Emergency herniotomy/hernioplasty after resuscitation
- c) Elective Lichtenstein mesh repair after 6 weeks
- d) Ultrasound-guided aspiration of hernia sac

Q69. A 17-year-old previously healthy boy presents to the emergency department at 3:00 AM with sudden onset severe left-sided scrotal pain that woke him from sleep. He reports associated nausea and two episodes of vomiting. There is no history of trauma, dysuria, urethral discharge, fever, or previous urinary tract infection.

On examination:

- Pulse: 112/min
- Temperature: 37°C
- Left hemiscrotum is swollen and erythematous
- Left testis is elevated and lies horizontally
- Cremasteric reflex is absent on the left side
- Prehn sign is negative

Color Doppler ultrasonography is not immediately available.

Which of the following is the most appropriate next step in management?

- a) Start intravenous antibiotics and review after 24 hours
- b) Obtain urgent MRI of the scrotum before intervention
- c) Immediate surgical exploration with bilateral orchidopexy
- d) Attempt needle aspiration of the tunica vaginalis



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q70. Golden period for salvaging torsion testis?

- a) 24 hours
- b) 12 hours
- c) 6 hours
- d) 48 hours

Q71. A patient presents with:



- Painless scrotal swelling
- Positive transillumination

Diagnosis?

- a) Hydrocele
- b) Varicocele
- c) Torsion
- d) Hernia

Q72. A 72-year-old multiparous woman presents with a 3-year history of a 6 cm full-thickness rectal prolapse that protrudes on defaecation and with walking. She has faecal incontinence affecting her quality of life. On examination, concentric mucosal folds are visible on the prolapsed segment. She is ASA II and medically fit for surgery. Anorectal physiology shows a weak internal and external anal sphincter. What is the MOST appropriate surgical treatment?

- a) Delorme's procedure — mucosal sleeve resection + plication of muscle wall
- b) Laparoscopic ventral mesh rectopexy (VMR) — posterior dissection avoided, preserving autonomic nerves, with mesh fixation to sacral promontory
- c) Thiersch wire procedure — anal cerclage to prevent prolapse



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- d) Conservative management with pelvic floor physiotherapy alone — surgery not indicated in elderly women

Q73. A 68-year-old woman with multiple comorbidities (COPD, heart failure, BMI 38) presents with a 7 cm full-thickness rectal prolapse that is permanently prolapsed and cannot be reduced. She is ASA IV and unfit for general anaesthesia. On examination, the prolapse is viable, non-ischaemic, and reducible with gentle manual pressure. What is the MOST appropriate surgical approach?

- a) Emergency laparoscopic ventral mesh rectopexy under general anaesthesia
- b) Altemeier's perineal rectosigmoidectomy under spinal/regional anaesthesia
- c) Thiersch wire procedure under local anaesthesia
- d) Manual reduction + conservative management with laxatives — surgery contraindicated in ASA IV

Q74. A 2-year-old girl presents with painless rectal prolapse. Her mother has noticed the rectal mucosa prolapsing up to 5cm outside the anus, especially during defaecation, and reducing spontaneously. No predisposing condition identified. What is the FIRST-LINE management?

- a) Immediate laparoscopic rectopexy (Sutureless Wells procedure)
- b) High-fibre diet, stool softeners, biofeedback + strapping of buttocks — conservative management first
- c) Thiersch wire procedure
- d) Delorme's mucosectomy

Q75. A 40-year-old woman presents with a painful blue-black, thrombosed perianal lump that appeared acutely 2 days ago. She is in severe pain. Examination shows a 2 cm thrombosed external haemorrhoid. What is the CORRECT management?

- a) Emergency haemorrhoidectomy — all thrombosed haemorrhoids require surgery
- b) If presenting within 72 hours: surgical excision under local anaesthesia — fastest pain relief; after 72 hours: conservative management (analgesia + warm sitz baths + stool softeners)
- c) IV antibiotics — treat as perianal abscess
- d) Rubber band ligation — most effective for thrombosed haemorrhoids

Q76. A 55-year-old woman with a long history of ulcerative colitis presents with an anal fistula. Examination shows a fistula tract passing through >50% of the external sphincter. MRI pelvis confirms a high trans-sphincteric fistula. What is the BEST surgical approach?

- a) Lay-open fistulotomy — suitable for all fistulae



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- b) Loose cutting seton — progressive division of sphincter muscle
- c) Ligation of intersphincteric fistula tract (LIFT) procedure or advancement flap — sphincter-preserving for complex fistulae
- d) Permanent colostomy as the only option

Q77. A 30-year-old man presents with a painless midline natal cleft pit with hair tufts. He develops acute abscess formation with pain, swelling, and discharge. Definitive management of pilonidal disease (after drainage of acute abscess)?



- a) Lay open the tract and allow secondary intention healing
- b) Wide excision with primary midline closure
- c) Karydakis flap or Bascom cleft-lift procedure (off-midline closure) — lowest recurrence rate
- d) Laser hair removal alone

Q78. A 45-year-old woman undergoes total thyroidectomy for multinodular goitre with bilateral nodules (Thy 4 right). Post-operatively she develops perioral tingling, Chvostek's sign, and carpopedal spasm. Corrected calcium is 1.7 mmol/L. What is the MOST appropriate management?

- a) Oral calcium supplements alone
- b) IV calcium gluconate 10 mL of 10% solution slowly + oral calcium 1500mg/day + alfacalcidol (Vitamin D analogue)
- c) Magnesium supplementation only
- d) IM PTH injection (teriparatide)

Q79. A 48-year-old woman undergoes total thyroidectomy.

Postoperatively she develops:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- Tingling
- Carpopedal spasm

ECG:

- Prolonged QT interval

Most likely diagnosis?

- a) Hyperkalemia
- b) Hypocalcemia
- c) Hypermagnesemia
- d) Hypoglycemia

Q80. A 32-year-old woman presents with a tender, erythematous, fluctuant mass in the right breast, 2 weeks postpartum, not responding to oral flucloxacillin for 5 days. USS shows a 3 cm abscess. What is the MOST appropriate management?

- a) Continue antibiotics for 2 more weeks
- b) USS-guided aspiration ± repeat aspiration; continue breastfeeding if possible
- c) Emergency mastectomy to prevent septicaemia
- d) IV antibiotics alone for 5 days then reassess

Q81. A 45-year-old woman presents with green, cheesy, bilateral multiduct nipple discharge. She is a smoker. USS shows periductal oedema. What is the MOST likely diagnosis?

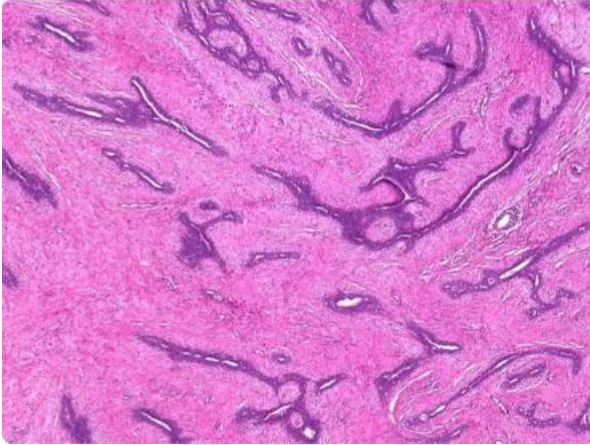
- a) Intraductal papilloma
- b) Duct ectasia (mammary duct ectasia)
- c) Galactorrhoea from prolactinoma
- d) Paget's disease of the nipple

Q82. A 28-year-old woman presents with:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- Mobile breast lump
- 2 cm
- Smooth surface
- Firm consistency

Ultrasound:

- Well-defined homogeneous lesion

FNAC:

- Benign epithelial and stromal elements

Most likely diagnosis?

- a) Fibroadenoma
- b) Phyllodes
- c) Carcinoma
- d) Breast abscess

Q83. A 28-year-old woman presents with a rapidly enlarging, painless, 6 cm lobulated breast mass over 3 months. USS shows a heterogeneous well-circumscribed solid mass. Core biopsy shows hypercellular spindle cell stroma with leaf-like projections. No atypia. What is the diagnosis and management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) Fibroadenoma — reassure, annual USS
- b) Benign phyllodes tumour — wide local excision with 1 cm clear margins
- c) Malignant phyllodes tumour — mastectomy + axillary clearance
- d) Invasive ductal carcinoma — neoadjuvant chemotherapy first

Q84. A 55-year-old chronic smoker presents with:

- Non-healing ulcer on lateral border of tongue
- Difficulty swallowing
- Referred otalgia

Biopsy demonstrates moderately differentiated squamous cell carcinoma.

Contrast-enhanced CT reveals:

- Primary lesion 3.5 cm
- Ipsilateral cervical node 4 cm
- No distant metastasis

Which factor has the greatest impact on long-term survival?

- a) Size of primary tumor
- b) Histological differentiation
- c) Presence of cervical lymph node metastasis
- d) Smoking history

Q85. A 60-year-old man presents with:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- Progressive dysphagia
- Hoarseness
- Weight loss

Upper GI endoscopy reveals post-cricoid carcinoma.

Which syndrome should be specifically asked about in history?

- a) MEN syndrome
- b) Plummer-Vinson syndrome
- c) Gardner syndrome
- d) Zollinger-Ellison syndrome

Q86. A 62-year-old man presents with progressive dysphagia. Barium swallow shows shouldering and irregular narrowing in lower esophagus. Endoscopy confirms carcinoma. Which symptom pattern best supports malignant obstruction?

- a) Dysphagia to solids and liquids from onset
- b) Dysphagia initially to solids then liquids
- c) Intermittent dysphagia only during stress
- d) Dysphagia relieved by nitrates

Q87. A 70-year-old patient presents with gastric outlet obstruction.

Endoscopy reveals pyloric narrowing.

Biopsy repeatedly negative.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

CT demonstrates:

- Thickened gastric wall
- Linitis plastica appearance

Most likely histology?

- a) Intestinal adenocarcinoma
- b) Diffuse signet-ring adenocarcinoma
- c) GIST
- d) Squamous carcinoma

Q88. A 55-year-old man undergoes CT abdomen for vague abdominal discomfort. Imaging shows a 7-cm exophytic gastric mass arising from the muscularis propria. Endoscopic biopsy is inconclusive. Immunohistochemistry shows CD117 positivity.

Most appropriate targeted drug?

- a) Rituximab
- b) Trastuzumab
- c) Imatinib
- d) Cetuximab

Q89. A 48-year-old man has recurrent multiple duodenal ulcers, chronic diarrhea, and epigastric pain refractory to proton pump inhibitors. Fasting serum gastrin is markedly elevated. Secretin stimulation test shows paradoxical rise in gastrin.

Most likely tumor location?

- a) Tail of pancreas only
- b) Gastrinoma triangle
- c) Spleen
- d) Left adrenal gland

Q90. A 45-year-old man with ulcerative colitis and primary sclerosing cholangitis presents with progressive jaundice, pruritus, and weight loss. MRCP shows multifocal biliary strictures with a dominant hilar stricture. CA 19-9 is elevated.

Most likely diagnosis?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Hepatocellular carcinoma
- b) Cholangiocarcinoma
- c) Gallbladder empyema
- d) Choledochal cyst

Q91. A 58-year-old man undergoes laparoscopic cholecystectomy for symptomatic gallstones.

Histopathology unexpectedly reveals:

- T1b adenocarcinoma
- Muscular layer involvement
- Negative margins

Best management?

- a) Observation
- b) Chemotherapy alone
- c) Radical cholecystectomy with liver wedge resection and lymphadenectomy
- d) Whipple procedure

Q92. A 72-year-old male presents with:

- Progressive constipation
- Pencil-thin stools
- Iron deficiency anemia

Colonoscopy reveals sigmoid growth.

Most likely molecular abnormality?

- a) RET mutation
- b) APC mutation
- c) VHL mutation
- d) MEN1 mutation

Q93. A 61-year-old man presents with left-sided colon obstruction. CT suggests sigmoid carcinoma. There is gross proximal colonic dilatation and unprepared bowel. He is unstable.

Best emergency operation?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Hartmann procedure
- b) Elective colonoscopy after 6 months
- c) Local excision
- d) Appendectomy

Q94. A 47-year-old patient with a gastric submucosal tumor undergoes biopsy. Immunohistochemistry shows DOG1 and CD117 positivity. Mitotic index is high and tumor size is 8 cm.

Most likely diagnosis?

- a) Gastric lymphoma
- b) Gastrointestinal stromal tumor
- c) Gastric adenocarcinoma
- d) Leiomyoma

Q95. A 60-year-old man presents with a change in bowel habit and rectal bleeding for 3 months. Colonoscopy shows a 3 cm sessile polyp in the sigmoid colon. Biopsy: adenocarcinoma invading the submucosa (T1). Staging CT is negative. Endoscopic mucosal resection (EMR) was incomplete with positive deep margin. What is the NEXT step?

- a) Reassure — endoscopic treatment attempted; repeat colonoscopy in 3 months
- b) Radical sigmoid colectomy — T1 colorectal cancer with positive deep margin after EMR
- c) Adjuvant chemotherapy — FOLFOX for T1 positive margin
- d) Radiotherapy to sigmoid colon

Q96. A 46-year-old patient presents with:

- Intermittent flushing
- Wheezing
- Diarrhea

Urinary 5-HIAA elevated.

Most likely diagnosis?

- a) GIST
- b) Carcinoid tumor
- c) VIPoma
- d) Gastrinoma



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q97. A 32-year-old man with MEN1 presents with recurrent peptic ulcers despite maximum PPI therapy. Fasting gastrin is 1200 pg/mL (normal <100). Secretin stimulation test shows >200 pg/mL rise in gastrin. Somatostatin receptor scintigraphy (Octreotide scan) shows a 1.5 cm lesion in the duodenum. Diagnosis and management?

- a) Helicobacter pylori gastritis — triple therapy
- b) Zollinger-Ellison syndrome (gastrinoma) — surgical resection of duodenal gastrinoma + long-term PPI
- c) Autoimmune gastritis — B12 supplementation
- d) Carcinoid tumour of the stomach — octreotide therapy only

Q98. A 50-year-old woman presents with a 3×2 cm ulcerated, indurated lesion at the anal verge. Biopsy shows squamous cell carcinoma of the anus. Staging: T2N0M0. What is the FIRST-LINE treatment?

- a) Abdominoperineal resection (APER) — standard surgical treatment for anal SCC
- b) Chemoradiotherapy — Nigro protocol (5-FU + mitomycin C + 45–54 Gy RT) — sphincter-preserving
- c) Wide local excision — adequate for T2 anal SCC
- d) Colostomy formation + palliative chemotherapy

Q99. A 61-year-old woman presents with progressive obstructive jaundice, pruritus, weight loss, and pale stools. CT shows a 2.8-cm perampullary tumor. Endoscopy reveals an ampullary mass. Biopsy confirms adenocarcinoma. No distant metastasis is seen.

Best curative operation?

- a) Segmental duodenal resection
- b) Pancreaticoduodenectomy
- c) Distal pancreatectomy
- d) Palliative biliary stenting only

Q100. A 54-year-old man undergoes CT for mucinous ascites. Laparoscopy shows gelatinous material throughout the peritoneal cavity and a ruptured appendiceal mucinous neoplasm.

Most appropriate term for this condition?

- a) Peritoneal carcinomatosis
- b) Pseudomyxoma peritonei
- c) Meigs syndrome



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

d) Krukenberg tumor

Q101. A 59-year-old man presents with left flank discomfort, weight loss, and intermittent hematuria. Examination reveals a non-reducible left-sided varicocele that does not disappear on lying down. Blood tests show hemoglobin 18.5 g/dL and serum calcium 12.1 mg/dL.

Most likely diagnosis?

- a) Benign prostatic hyperplasia
- b) Renal cell carcinoma
- c) Transitional cell carcinoma of ureter
- d) Chronic prostatitis

Q102. A 62-year-old chronic smoker presents with painless gross hematuria for 3 weeks. He denies fever, flank pain, colicky pain, or dysuria. Urine microscopy shows RBCs but no casts. Ultrasound shows a 3-cm papillary lesion arising from the lateral wall of the urinary bladder. CECT abdomen shows no upper tract mass.

What is the most appropriate next step?

- a) Empirical antibiotics for presumed cystitis
- b) TURBT with histopathological staging
- c) Radical cystectomy without biopsy
- d) ESWL followed by cystoscopy

Q103. A 68-year-old man undergoes flexible cystoscopy for haematuria. A 1.5 cm sessile papillary tumour is seen at the right lateral wall. TURBT histology: T1G3 (high-grade, lamina propria invasion). Re-TURBT at 4–6 weeks shows residual T1G3. BCG induction course is completed (6 weeks). Which of the following is the NEXT management step?

- a) Continue BCG maintenance for 3 years (SWOG protocol)
- b) Repeat TURBT only — no further treatment
- c) Radical cystectomy — T1G3 with incomplete BCG response strongly considered for early radical cystectomy
- d) Gemcitabine + cisplatin chemotherapy — superior to BCG for T1G3



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q104. A 70-year-old man has PSA 28 ng/mL, DRE shows a hard nodule in the right lobe. TRUS-guided biopsies (12 cores): Gleason 4+3=7 (Grade Group 3) in 6/12 cores. Staging: MRI shows T2c disease (bilateral involvement, no extraprostatic extension). Bone scan negative. Treatment options?

- a) Active surveillance — Gleason 7 can be monitored without treatment
- b) Radical prostatectomy OR radical radiotherapy + androgen deprivation therapy — both curative options for intermediate-high risk localised prostate cancer
- c) Androgen deprivation therapy (ADT) alone — curative for localised disease
- d) Watchful waiting only — for all T2 prostate cancer

Q105. Most common site of metastasis from prostate cancer?

- a) Liver
- b) Lung
- c) Bone
- d) Brain

Q106. A 62-year-old man has a PSA of 7.2 ng/mL on routine screening. His DRE reveals a hard nodule in the right lobe of the prostate. He is otherwise asymptomatic. MRI of the prostate (mpMRI) shows a PI-RADS 5 lesion in the right peripheral zone. Targeted biopsy confirms Gleason 4+3=7 (Grade Group 3) adenocarcinoma in 4 of 6 cores from the right lobe. Staging CT chest/abdomen/pelvis and bone scan show no metastases. Clinical stage: T2cN0M0. What is the MOST appropriate management?

- a) Active surveillance — Gleason 4+3 can be safely monitored without treatment
- b) Androgen deprivation therapy (ADT) alone — curative for localised prostate cancer
- c) Radical prostatectomy OR radical radiotherapy + short-course ADT (6 months) — both are curative options for intermediate-high risk localised prostate cancer
- d) Watchful waiting — appropriate for all men with localised prostate cancer

Q107. A 26-year-old man presents 18 months after completing 3 cycles of BEP for Stage IIA good-prognosis NSGCT (embryonal carcinoma). He was in complete remission with normal markers and CT. He now presents with rising markers: AFP 340 ng/mL (was normal), β -hCG 28 IU/L (was normal), LDH 420 U/L. CT shows new 2.5 cm retroperitoneal lymph nodes and two new pulmonary nodules. He has relapsed. What is the MOST appropriate salvage treatment?

- a) Repeat 3 cycles of BEP — same regimen as first-line
- b) Salvage chemotherapy with TIP (paclitaxel + ifosfamide + cisplatin) OR VeIP (vinblastine + ifosfamide + cisplatin) for 4 cycles — then high-dose chemotherapy + autologous stem cell transplant (HDCT/ASCT) in selected cases

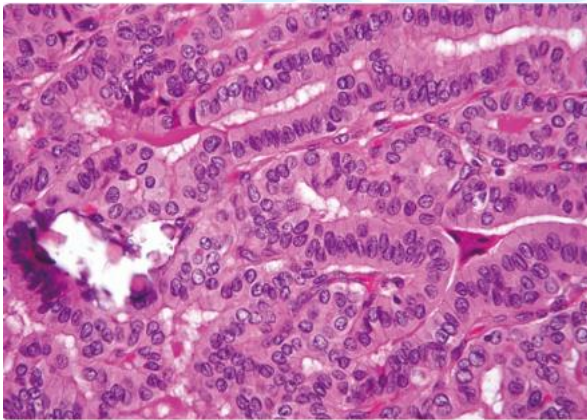


MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- c) Surgical resection of retroperitoneal nodes + lung wedge resection — no further chemotherapy needed
- d) Pembrolizumab (PD-1 inhibitor) — immunotherapy is first-line for relapsed GCT

Q108. A 40-year-old woman presents with a 3 cm solitary thyroid nodule. She has no cervical lymphadenopathy. TSH is normal. USS shows a hypoechoic nodule with irregular margins and microcalcifications (U5 on BTA classification). FNAC reports Thy 5 (malignant — consistent with papillary thyroid carcinoma). What is the MOST appropriate surgical management?



- a) Hemithyroidectomy only — sufficient for papillary thyroid carcinoma <4cm
- b) Total thyroidectomy + central compartment lymph node dissection (level VI) ± radioiodine ablation
- c) Radioiodine alone — avoids surgery
- d) Total thyroidectomy + prophylactic modified radical neck dissection (levels II–V)

Q109. A 60-year-old woman presents with a palpable thyroid mass, hoarseness, and a positive serum calcitonin of 890 pg/mL (normal <10). Core biopsy: medullary thyroid carcinoma. RET proto-oncogene mutation: RET M918T (MEN2B). CT shows bilateral cervical lymphadenopathy. What is the correct management?

- a) Radioiodine ablation — effective for medullary thyroid carcinoma
- b) Total thyroidectomy + central neck dissection (level VI) + bilateral lateral neck dissection (levels II–V) — then exclude pheochromocytoma pre-operatively
- c) Thyroidectomy alone without neck dissection — calcitonin levels guide further treatment
- d) Vandetanib (RET inhibitor) only — avoids surgery

Q110. A 50-year-old woman post wide local excision for IDC has final histology: 2cm IDC, Grade 2, ER+, PR+, HER2-, Ki-67 18%, margins clear (>2mm), 1/3 sentinel nodes positive, no extracapsular extension. She meets Z0011 criteria. What adjuvant treatment is planned?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Whole breast radiotherapy + adjuvant endocrine therapy (aromatase inhibitor) — no chemotherapy
- b) Chemotherapy alone — endocrine therapy not needed for PR+ tumours
- c) Trastuzumab for 12 months — HER2+ treatment
- d) Radiotherapy alone — no endocrine therapy needed for Grade 2 tumours

Q111. A 42-year-old woman presents with a painless breast lump noticed 4 months ago. She has no family history of breast cancer. Examination reveals:

- 2.8 cm hard irregular lump
- Upper outer quadrant
- Not fixed to chest wall
- Single mobile ipsilateral axillary node

Mammography:

- Spiculated lesion
- BI-RADS 5

Core biopsy:

- Invasive ductal carcinoma
- ER positive
- PR positive
- HER2 negative

PET-CT:

- No distant metastasis

What is the most appropriate initial management?

- a) Modified radical mastectomy immediately
- b) Wide local excision without axillary surgery
- c) Breast-conserving surgery with sentinel lymph node biopsy
- d) Tamoxifen alone

Q112. Lymph nodes are divided into axillary levels based on their relationship to which structure?

- a) Pectoralis major muscle
- b) Pectoralis minor muscle
- c) Serratus anterior muscle
- d) Subscapularis muscle



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q113. Which of the following is the MOST common histological subtype of breast cancer?

- a) Invasive lobular carcinoma (ILC)
- b) Invasive ductal carcinoma — No Special Type (NST)
- c) Mucinous carcinoma
- d) Tubular carcinoma

Q114. A 35-year-old man is brought to the ED following a road traffic accident. On assessment: he opens his eyes only when you call his name loudly, he is confused and disoriented but produces full sentences, and he withdraws his right hand when you press firmly on his nail bed. His left side shows no response to painful stimuli. What is his GCS score, and what is the MOST appropriate immediate airway management decision?

- a) GCS 12
- b) GCS 11 (E3V4M4)
- c) GCS 10 (E3V4M3)
- d) GCS 13 (E3V4M6)

Q115. A 45-year-old woman with known alcohol dependency is found unconscious in the street. She has a strong smell of alcohol. Her GCS is assessed as: eyes do not open to any stimulus (E1), she makes incomprehensible groaning sounds (V2), and her arms extend and pronate in response to sternal rub (M2). Her GCS is therefore 5. Her blood glucose is 3.1 mmol/L. CT head is performed. What does a GCS of 5 indicate clinically and what is the MOST important immediate intervention?

- a) GCS 5 indicates mild head injury — oral glucose and observe
- b) GCS 5 (E1V2M2) indicates severe brain injury or metabolic cause; airway must be secured with intubation
- c) GCS 5 indicates moderate head injury — nasogastric tube
- d) GCS 5 with alcohol smell = alcohol intoxication only — no intervention needed

Q116. A patient undergoes total thyroidectomy.

Six hours later he develops:

- Neck swelling
- Stridor
- Respiratory distress



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Most likely diagnosis?

- a) Recurrent laryngeal nerve palsy
- b) Hypocalcemia
- c) Expanding neck hematoma
- d) Thyroid storm

Q117. During massive transfusion (>10 units pRBC), patient develops perioral tingling, hypotension, prolonged QT, corrected Ca^{2+} 1.6. Cause?

- a) Hyperkalaemia from stored blood
- b) Citrate anticoagulant chelating ionised calcium
- c) Hypothermia reducing calcium absorption
- d) Dilutional coagulopathy

Q118. ATLS Class II haemorrhagic shock — which of the following is CORRECT?

- a) Blood loss 5–15%, HR <100, normal BP, no resuscitation needed
- b) Blood loss 15–30%, HR >100, normal BP, may need crystalloid resuscitation
- c) Blood loss 30–40%, HR >120, decreased BP, needs blood transfusion
- d) Blood loss >40%, HR >140, no BP, emergency surgery

Q119. In massive transfusion protocol, the recommended ratio of blood products is:

- a) 1:1:1 (pRBC:FFP:Platelets) — PROPPR trial
- b) 2:1:1 (pRBC:FFP:Platelets)
- c) 1:2:1 (pRBC:FFP:Platelets)
- d) pRBC only until haemostasis achieved, then FFP

Q120. Which of the following is NOT a complication of massive blood transfusion?

- a) Hypocalcaemia from citrate toxicity
- b) Hyperkalaemia from aged stored blood
- c) Hypothermia from cold blood
- d) Hypermagnesaemia



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q121. EDT (Emergency Department Thoracotomy) indications — which is CORRECT?

- a) EDT contraindicated if >10 minutes pulseless in penetrating trauma
- b) EDT indicated in penetrating chest trauma with <15 minutes of pulselessness
- c) EDT indicated in all blunt trauma with any pulselessness regardless of time
- d) EDT only after failed pericardiocentesis

Q122. A 34-year-old male is brought to the emergency department following a high-speed road traffic collision. He was not wearing a seatbelt. On arrival:

- BP 80/40 mmHg
- Pulse 132/min
- Respiratory rate 30/min
- Oxygen saturation 88%

Clinical examination reveals:

- Distended neck veins
- Hyperresonance on percussion over the right hemithorax
- Absent breath sounds on the right side
- Tracheal deviation to the left

FAST examination is negative for intraperitoneal fluid.

Which of the following is the most appropriate immediate intervention?

- a) Portable chest radiograph
- b) CT thorax with contrast
- c) Needle thoracostomy followed by tube thoracostomy
- d) Emergency thoracotomy

Q123. A 22-year-old man is stabbed in the right side of the chest. On arrival: HR 130, BP 90/60, decreased breath sounds on the right, dull to percussion on the right. His SpO₂ is 89%. CXR shows opacification of the right hemithorax with >1500 mL blood estimated. What is the diagnosis and management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) Tension pneumothorax — needle decompression 2nd ICS MCL right
- b) Massive haemothorax — chest drain (28–32F) insertion RIGHT side + blood transfusion; thoracotomy if drainage >1500 mL initially or >200 mL/hour for 2–4 hours
- c) Pulmonary contusion — high-flow O₂ and observation
- d) Haemopneumothorax — chest drain left side

Q124. Most commonly injured solid organ in penetrating trauma?

- a) Spleen
- b) Liver
- c) Kidney
- d) Pancreas

Q125. A patient develops:

- Flail chest

Definition?

- a) One fractured rib
- b) Two consecutive ribs fractured
- c) Three or more adjacent ribs fractured in two places
- d) Sternum fracture

Q126. Best indicator of adequate resuscitation?

- a) Pulse
- b) Blood pressure



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- c) Urine output
- d) Respiratory rate

Q127. ATLS sequence?

- a) CABDE
- b) ABCDE
- c) BACDE
- d) ACBDE

Q128. A 30-year-old motorcyclist is brought to the ED following a collision. He is haemodynamically unstable (BP 70/40, HR 145). FAST shows free fluid in Morrison's pouch, spleno-renal recess, and pelvis. His abdomen is rigid. He is taken immediately to theatre. Which of the following describes the CORRECT damage control surgery (DCS) sequence?

- a) Full anatomical repair of all injuries in one prolonged operation
- b) Phase I: haemorrhage control + contamination control + temporary closure → Phase II: ICU resuscitation → Phase III: definitive repair 24–48h
- c) Non-operative management with angioembolisation alone
- d) Exploratory laparotomy with complete bowel resection and anastomosis in one stage

Q129. A 70-kg adult presents with 40% TBSA burns.

Using Parkland Formula:

Fluid requirement during first 24 hours?

- a) 5600 mL
- b) 8400 mL
- c) 11200 mL
- d) 14000 mL

Q130. A 35-year-old man sustains 35% TBSA flame burns involving the anterior trunk, both arms, and face. He is resuscitated with Parkland formula (Hartmann's solution). On day 3, burn wound assessment shows: face — superficial partial thickness (pink, painful, blistered), arms — deep partial thickness (pale, reduced sensation, fixed staining), anterior trunk — full thickness (leathery, insensate, white-brown eschar). Which burns require skin grafting and which can be managed conservatively?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) All burns >10% TBSA require immediate skin grafting regardless of depth
- b) Superficial partial thickness burns heal spontaneously — deep partial thickness and full thickness burns require skin grafting; face managed conservatively with antimicrobial dressings initially
- c) Full thickness burns only require grafting — all partial thickness burns heal without surgery
- d) Skin grafting is contraindicated in the first 2 weeks — all burns managed with silver dressings for 3 weeks first

Q131. A 65-year-old man with a 30-pack-year smoking history presents with cramping pain in his right calf after walking 200 metres, relieved by rest. ABPI (ankle-brachial pressure index) is 0.62 on the right. What is the diagnosis and FIRST-LINE management?

- a) Critical limb ischaemia — urgent revascularisation
- b) Intermittent claudication (Fontaine Stage IIb) — supervised exercise programme + smoking cessation + antiplatelet + statin therapy
- c) Acute limb ischaemia — emergency embolectomy
- d) Deep vein thrombosis — anticoagulation

Q132. A 70-year-old man is found to have an asymptomatic abdominal aortic aneurysm (AAA) on incidental USS. Diameter is 5.3 cm. He is otherwise fit (ASA II). What is the MOST appropriate management?

- a) Urgent elective repair — 5.0 cm threshold for repair
- b) Surveillance USS every 3 months — 5.3 cm does not yet meet repair threshold
- c) Elective repair (EVAR or open) — threshold is 5.5 cm in men, already reached risk/benefit crossover
- d) Conservative management with antihypertensives only — all asymptomatic AAA

Q133. A 75-year-old woman with AF presents with sudden onset severe left leg pain, pallor, and pulselessness distal to the left groin. The limb is cold and numb. She has no prior peripheral vascular disease. The onset was 2 hours ago. What is the MOST likely cause and immediate management?

- a) Acute limb ischaemia from thrombosis — IV heparin and urgent vascular review
- b) Acute limb ischaemia from embolism (AF source) — IV heparin + emergency Fogarty catheter embolectomy
- c) DVT — IV anticoagulation with LMWH
- d) Critical limb ischaemia from chronic atherosclerosis — urgent angioplasty



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

Q134. A 70-year-old woman has varicose veins in the long saphenous distribution with skin changes (lipodermatosclerosis) and a healed medial malleolar ulcer. ABPI is 0.95. Duplex scan confirms long saphenous vein (LSV) incompetence at the sapheno-femoral junction. What is the MOST appropriate treatment?

- a) Class II compression stockings alone — sufficient for all varicose veins
- b) Endovenous thermal ablation (EVLA or RFA) of the LSV + foam sclerotherapy for residual tributaries — preferred over surgery
- c) Open surgery (Trendelenburg's procedure + stripping of LSV) only
- d) Observation — varicose veins do not require treatment unless actively bleeding

Q135. A 68-year-old diabetic smoker presents with severe pain in the right foot for 6 months. Initially the pain occurred while walking approximately 200 meters but now wakes him from sleep. He obtains temporary relief by hanging the leg off the bed.

Examination reveals:

- Absent dorsalis pedis pulse
- Absent posterior tibial pulse
- Dependent rubor
- Elevation pallor
- Dry gangrene of the great toe

Ankle-brachial index (ABI): 0.28

Which of the following best describes his current stage of disease?

- a) Intermittent claudication
- b) Acute limb ischemia
- c) Critical limb ischemia
- d) Chronic venous insufficiency

Q136. A 65-year-old woman presents with:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- Long-standing varicose veins
- Medial malleolar ulcer
- Hyperpigmentation
- Lipodermatosclerosis

Most likely etiology?

- a) Arterial insufficiency
- b) Venous hypertension
- c) Vasculitis
- d) Diabetic neuropathy

Q137. A 60-year-old man presents with chronic leg ulcer on the medial aspect of the right ankle (gaiter area), with surrounding lipodermatosclerosis and haemosiderin deposition. ABPI is 1.0 (normal). Duplex scan confirms deep venous insufficiency. What is the MOST appropriate management?

- a) Arterial bypass surgery — ABPI 1.0 indicates arterial disease
- b) Four-layer compression bandaging (40 mmHg at ankle) + wound care — first-line for venous leg ulcers
- c) Split-skin grafting immediately without compression
- d) Systemic corticosteroids to reduce inflammation

Q138. A 58-year-old diabetic presents with:

- Non-healing plantar ulcer
- Loss of vibration sense



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- Loss of ankle jerk
- Warm foot
- Bounding pulses

Most likely mechanism?

- a) Arterial ischemia
- b) Neuropathic diabetic foot
- c) Vasculitis
- d) Buerger disease

Q139. A 28-year-old man presents with a 3 cm swelling in the right parotid region that has been present for 6 months. It is soft, fluctuant, and located anterior to the tragus. FNA shows lymphocytes and squamous cells with cholesterol crystals. No associated lymphadenopathy. Diagnosis?

- a) Pleomorphic adenoma — most common parotid tumour
- b) First branchial cleft cyst (Work Type II) — in or adjacent to the parotid, lined by squamous epithelium, contains cholesterol crystals
- c) Mucoepidermoid carcinoma — most common malignant parotid tumour
- d) Warthin's tumour (papillary cystadenoma lymphomatosum)

Q140. A 48-year-old woman presents with a slowly enlarging swelling in the left parotid region for the last 8 years. Initially it was pea-sized but has gradually enlarged to approximately 5 cm. There is no pain, facial weakness, trismus, weight loss, or cervical lymphadenopathy.

MRI demonstrates:

- Well-circumscribed lesion
- Located in superficial lobe of parotid
- No deep lobe extension
- No perineural invasion

FNAC reveals:

- Epithelial cells
- Myxochondroid stroma
- No atypia

Which of the following is the most appropriate definitive management?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Observation with annual MRI because malignant transformation is rare and surgery carries facial nerve risk
- b) Enucleation of the tumor alone to preserve maximum parotid tissue
- c) Superficial parotidectomy with preservation of facial nerve
- d) Radical parotidectomy with facial nerve sacrifice

Q141. A 40-year-old construction worker sustains a crush injury to the leg. Six hours later he develops severe pain despite analgesics. The leg is tense and swollen. Pain is markedly worsened by passive dorsiflexion of the toes. Distal pulses are palpable.

Most appropriate next step?

- a) Reassure because pulses are present
- b) Elevate limb tightly and apply compression bandage
- c) Emergency fasciotomy
- d) Wait for sensory loss before intervention

Q142. A 46-year-old diabetic patient presents with rapidly progressive pain and swelling of the perineum. Examination shows fever, tachycardia, erythema, edema, crepitus, foul-smelling discharge, and skin discoloration. Labs show leukocytosis, acidosis, and elevated creatinine.

Most appropriate management?

- a) Broad-spectrum antibiotics only
- b) Immediate wide surgical debridement with broad-spectrum antibiotics
- c) Wait for MRI confirmation before intervention
- d) Incision and drainage under local anesthesia only

Q143. A 50-year-old with short bowel syndrome (100 cm residual jejunum, no ileocaecal valve) requires long-term nutritional support. Which vitamin deficiency is MOST likely after 12 months?

- a) Vitamin C deficiency (scurvy)
- b) Vitamin B12 deficiency
- c) Vitamin D deficiency
- d) Vitamin K deficiency

Q144. A Kocher's hemostatic forceps is identified by:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026



- a) Triangular fenestrae and non-traumatic tip
- b) Opposing tooth on tip and longitudinal serrations throughout the blades
- c) Oval fenestrae without teeth
- d) Curved blades with smooth tips only

Q145. Identify the correct use of a Ryle's tube (nasogastric tube):

- a) It is a wide-bore 24F tube used for gastric decompression
- b) It is a fine-bore 8–12F tube used for feeding and gastric decompression
- c) It is used only for post-operative gastric decompression, not feeding
- d) Ryle's tube is always radio-opaque and self-positioning

Q146. Which of the following drains is a CLOSED SUCTION drain?



- a) Penrose drain — flat, passive, latex drain
- b) Corrugated drain — passive, open system
- c) Jackson-Pratt (JP) drain — closed suction drain with bulb reservoir
- d) Sump drain — open double-lumen drain

Q147. The LIGACLIP (metal haemostatic clip) used in laparoscopic surgery is made of:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

- a) Stainless steel — MRI incompatible
- b) Titanium — MRI compatible, permanent, used for cystic artery/duct ligation
- c) Absorbable polymer — temporary clips only
- d) Chromium alloy — always radio-opaque

Q148. A 45-year-old woman presents with a 5 cm firm, deep soft tissue mass in her thigh attached to deep fascia. MRI shows a well-defined mass with homogeneous signal, no necrosis, no vascular encasement. Core biopsy: myxoid liposarcoma. Staging: no metastases. Treatment?

- a) Observation — liposarcoma is always low grade and indolent
- b) Wide local excision with 1–2 cm clear margins + adjuvant radiotherapy (for large/high-grade tumours)
- c) Amputation — all liposarcomas require amputation
- d) Chemotherapy alone — liposarcomas are chemotherapy-sensitive tumours

Q149. A 28-year-old woman develops a wound dehiscence on post-op day 7 after caesarean section. The wound edges are separated with salmon-coloured serosanguinous discharge. The fascia is intact on probing. How should this wound be managed?



- a) Return to theatre for mass closure immediately
- b) Wash the wound, apply vacuum-assisted closure (VAC therapy), and allow secondary intention healing or delayed primary closure when wound is clean
- c) Antibiotics only
- d) Skin staples immediately

Q150. A 19-year-old man sustains a closed head injury in a rugby match. He was briefly unconscious for 2 minutes then recovered. In the ED his GCS is 15. He has a headache and reports amnesia for the event. Over the next 3 hours his GCS drops to 13, then to 9. CT head shows a



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

biconvex (lens-shaped) hyperdense collection in the right temporal region with 8mm midline shift.
What is the diagnosis, and how does the GCS trajectory guide the TIMING of surgical intervention?

- a) Acute subdural haematoma — urgent craniotomy only if GCS drops below 3
- b) Extradural haematoma (EDH) — the classic lucid interval followed by GCS deterioration is the hallmark; GCS deterioration is a surgical emergency requiring immediate craniotomy/burr hole
- c) Diffuse axonal injury — GCS trajectory is irrelevant; MRI is required before any decision
- d) Intracerebral haemorrhage — conservative management with antihypertensives

ANSWERS

1	b	31	c	61	b	91	c	121	b
2	b	32	b	62	c	92	b	122	c
3	b	33	b	63	a	93	a	123	b
4	b	34	c	64	b	94	b	124	b
5	b	35	b	65	b	95	b	125	c
6	b	36	b	66	b	96	b	126	c
7	b	37	c	67	b	97	b	127	b
8	b	38	b	68	b	98	b	128	b
9	b	39	b	69	c	99	b	129	c
10	b	40	b	70	c	100	b	130	b
11	a	41	b	71	a	101	b	131	b
12	b	42	b	72	b	102	b	132	c
13	b	43	c	73	b	103	c	133	b
14	b	44	b	74	b	104	b	134	b
15	c	45	b	75	b	105	c	135	c
16	b	46	c	76	c	106	c	136	b
17	b	47	b	77	c	107	b	137	b
18	b	48	c	78	b	108	b	138	b
19	a	49	b	79	b	109	b	139	b
20	c	50	b	80	b	110	a	140	c
21	b	51	b	81	b	111	c	141	c
22	a	52	a	82	a	112	b	142	b
23	c	53	b	83	b	113	b	143	b
24	b	54	b	84	c	114	b	144	b
25	b	55	b	85	b	115	b	145	b
26	c	56	c	86	b	116	c	146	c
27	c	57	c	87	b	117	b	147	b
28	b	58	b	88	c	118	b	148	b
29	b	59	b	89	b	119	a	149	b
30	b	60	b	90	b	120	d	150	b



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

SURGERY SUBJECT WISE TEST , DATED ON 05th JUNE 2026

